SYNTHETIC APPLICATION DOSSIER

Rural Care Access Pilot

Funding opportunity and draft project materials for a grant readiness review.

Applicant: Cedar Valley Community Health (CVCH)
Sponsor: Northstar Community Research Foundation (fictional)
Opportunity: Rural Access Pilot Awards 2027 | NCRF-RAP-2027-01

Prepared as of: October 28, 2026
Narrative funding request: $198,000
Status: Working application packet; no submission has occurred.

About this packet

Every organization, person, program, and rule in this packet is invented. The documents represent a realistic working application with incomplete information and differing drafts. They are intended for software testing only, not for funding or compliance decisions.

Document register

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Source labels and numbered sections are included so reviews can cite precise evidence. Referenced attachments are not included unless reproduced in this packet.

DOCUMENT A / SPONSOR NOTICE V1.0 / OCTOBER 1, 2026

Funding opportunity: award rules

A1. Purpose

The Rural Access Pilot Awards support small evaluations of care-navigation services in rural communities. Projects must measure feasibility and access outcomes, describe an evaluation method, and explain how findings will be shared with participating communities.

A2. Applicant and investigator eligibility

The lead applicant must be a United States nonprofit organization with a current tax-exempt determination letter and at least two years of community health operations. A rural clinic or community health partner must participate. The principal investigator (PI) must be an employee of the lead applicant at submission. Contractors may serve as co-investigators but may not serve as PI. The applicant must document the PI employment relationship in its institutional endorsement.

A3. Award amount and project period

Applicants may request up to **$200,000 in total sponsor funds, including indirect costs**. The project period is **January 1 through December 31, 2027 (12 months)**. Pre-award costs are not reimbursable. Activities proposed outside the project period require a revised work plan; an extension is not assumed.

A4. Budget rules

Category	Sponsor rule
Indirect costs	Maximum 10% of direct costs, excluding equipment. No other exclusions apply for this program.
Equipment	Any single item costing $5,000 or more is equipment. Each such item requires written sponsor approval before submission; attach the approval to the budget justification.
Personnel and consultants	Identify roles, effort or hours, rates, and the basis for each estimate. Include a narrative justification for every category.
Cost sharing	Not required and does not improve review scores. Any voluntary commitment must have written approval from an authorized institutional official.

A5. Human participants and data

If the project involves human participants, include a human-subjects plan at application. Institutional review board (IRB) approval or a documented exemption must be obtained before recruitment or collection of participant data. Approval is not required at submission. Describe consent, privacy safeguards, and storage of identifiable information.

Funding opportunity: submission

A6. Deadline and submission method

The full application must be received through the sponsor portal by **November 19, 2026, at 5:00 p.m. Eastern Time**. Late submissions are not accepted. There is no letter-of-intent stage. Submission must be completed by an authorized institutional official; a PI cannot submit on behalf of the institution without that authority.

A7. Required application components

Component	Requirement
Project narrative	Maximum 6 pages: need, objectives, methods, evaluation, timeline, and dissemination.
Budget and justification	Completed budget worksheet plus a narrative justification of up to 2 pages. Include applicable equipment approvals.
PI biosketch	Maximum 2 pages; describe qualifications and current institutional appointment.
Partner commitment	Signed letter from at least one participating rural clinic or community health partner, identifying responsibilities.
Institutional endorsement	Signed by an authorized institutional official. Confirm eligibility, PI employment, resources, and any voluntary cost sharing.
Tax-exempt determination	Copy of the current determination letter for the lead applicant.
Human-subjects plan	Required when human participants are involved; address review status, consent, privacy, and data storage.

A8. Review criteria

Applications are assessed on community need (25%), feasibility (30%), evaluation quality (25%), and organizational readiness (20%). Claims of prior outcomes should identify the underlying report or dataset. An unsupported projection should be identified as a target or estimate, not an established result.

A9. Sponsor clarification record

This notice is the complete sponsor guidance available in the packet. No amendments, deadline extensions, equipment approvals, or eligibility exceptions are included.

End of fictional sponsor notice. Applicant drafts on subsequent pages are separate sources and do not amend these requirements.

DOCUMENT B / APPLICANT NARRATIVE V0.3 / OCTOBER 23, 2026

Project narrative summary

B1. Project and team

Project: Rural Care Access Pilot. **Lead applicant:** Cedar Valley Community Health. **Proposed PI:** Dr. Maya Ellison. **Partner:** Pine Creek Rural Clinic. CVCH reports operating a community navigation service since 2021 and describes itself as a tax-exempt nonprofit.

B2. Need and proposed approach

The project will offer navigation calls and appointment reminders to 120 adults who report transportation or scheduling barriers. Pine Creek Rural Clinic will refer eligible adults and help compare appointment attendance before and after navigation. Participants will complete short surveys at enrollment and at three months.

B3. Intended outcomes

The team targets a 20% relative reduction in missed appointments and at least 90 completed follow-up surveys. The draft states: "Our earlier pilot reduced missed appointments by 35%." No citation, sample size, baseline, or underlying report accompanies that statement. Analysis will compare attendance records before and after enrollment; a comparison group has not been specified.

B4. Work plan and dates

Phase	Draft schedule
Clinic screening and recruitment	Begin December 15, 2026
Navigation and participant follow-up	January through December 2027
Final analysis and dissemination	January through June 2028
Overall duration	18 months; January 2027 through June 2028

B5. Resources and requested funds

The narrative requests **$198,000** from the sponsor. It also describes a **$20,000 CVCH cash contribution** to expand recruitment. The team proposes two portable assessment devices and consultant support for evaluation. Detailed cost assumptions are maintained in the finance worksheet.

B6. Human-subjects approach

Clinic staff will share names, phone numbers, and appointment histories with the project team. The draft anticipates written consent and a restricted-access institutional drive. Retention duration and responsibility for access reviews are not specified. The team expects the project to qualify for an IRB exemption but has not received a determination.

DOCUMENT C / FINANCE WORKSHEET V0.2 / OCTOBER 25, 2026

Proposed budget worksheet

All amounts below are requested from the sponsor. The voluntary applicant contribution described in the narrative is separate from this worksheet. Estimates are provisional.

Cost item	Calculation / basis	Sponsor request
Personnel	PI $36,000; navigator $48,000; analyst $12,000	$96,000
Fringe benefits	30% of personnel costs	$28,800
Travel	Local outreach and clinic travel	$12,000
Supplies	Participant materials and office supplies	$18,000
Equipment	2 portable assessment devices at $12,000 each	$24,000
Consultant	Evaluation specialist: 200 hours at $100/hour	$20,000
Total direct costs	Sum of the six categories above	$198,800
Indirect costs	10% x ($198,800 - $24,000)	$17,480
TOTAL SPONSOR REQUEST	Direct plus indirect costs	$216,280

C1. Finance notes

The budget uses the sponsor indirect-cost formula. Salary estimates require confirmation of appointment status and effort. Travel and supply estimates are planning figures; detailed calculations have not yet been drafted.

C2. Equipment procurement

The team has identified two assessment devices, each costing $12,000. A vendor quote has been requested. Sponsor permission for these purchases is not on file. The devices are included in the sponsor request above.

C3. Applicant contribution

A $20,000 cash contribution is mentioned in the project narrative. Finance has not approved an account or source of funds. This contribution is not included in the sponsor totals above.

C4. Supporting documentation

The separate narrative budget justification is still to be written. No signed budget approval, equipment authorization, or cost-sharing authorization is attached.

DOCUMENT D / INTERNAL NOTES / OCTOBER 28, 2026

Readiness notes and file register

D1. Internal planning notes

Coordinator schedule: Submit the application on November 20, 2026, at 5:00 p.m. Eastern Time. A finance review meeting is penciled in for November 18. No institutional routing deadline has been confirmed.

Personnel: Dr. Ellison currently provides services under an independent consultant agreement. HR is exploring a part-time employee appointment effective December 1, 2026; no appointment letter has been issued. The team has not requested a sponsor eligibility exception.

Partner: Pine Creek Rural Clinic has expressed interest by phone. Its director has not signed a commitment letter. Referral volume and access to appointment records remain under discussion.

IRB: A protocol draft is in preparation. Nothing has been submitted to the IRB, and no exemption determination or approval has been received. The December recruitment date remains in the narrative draft.

D2. File register at packet assembly

Item	Status recorded by coordinator
Project narrative	Summary reproduced on page 4; complete 6-page narrative not yet assembled.
Budget worksheet	Reproduced on page 5; finance draft, not approved.
Budget justification	Not drafted.
PI biosketch	Requested from PI; not received.
Partner commitment letter	Unsigned draft planned; no signed letter received.
Institutional endorsement	Not routed; authorized signing official not identified in these notes.
Tax-exempt determination	Staff believe it is in the corporate archive; not included in this packet.
Human-subjects plan	Partial description in narrative; separate plan not assembled.
Prior pilot outcomes report	Not included; team is looking for supporting attendance data.

D3. Open administrative items

Confirm sponsor deadline, applicant eligibility documentation, PI appointment, partner responsibilities, a feasible project schedule, and the final funding request. Identify an institutional reviewer and signing official. No application approval or portal submission receipt is on file.